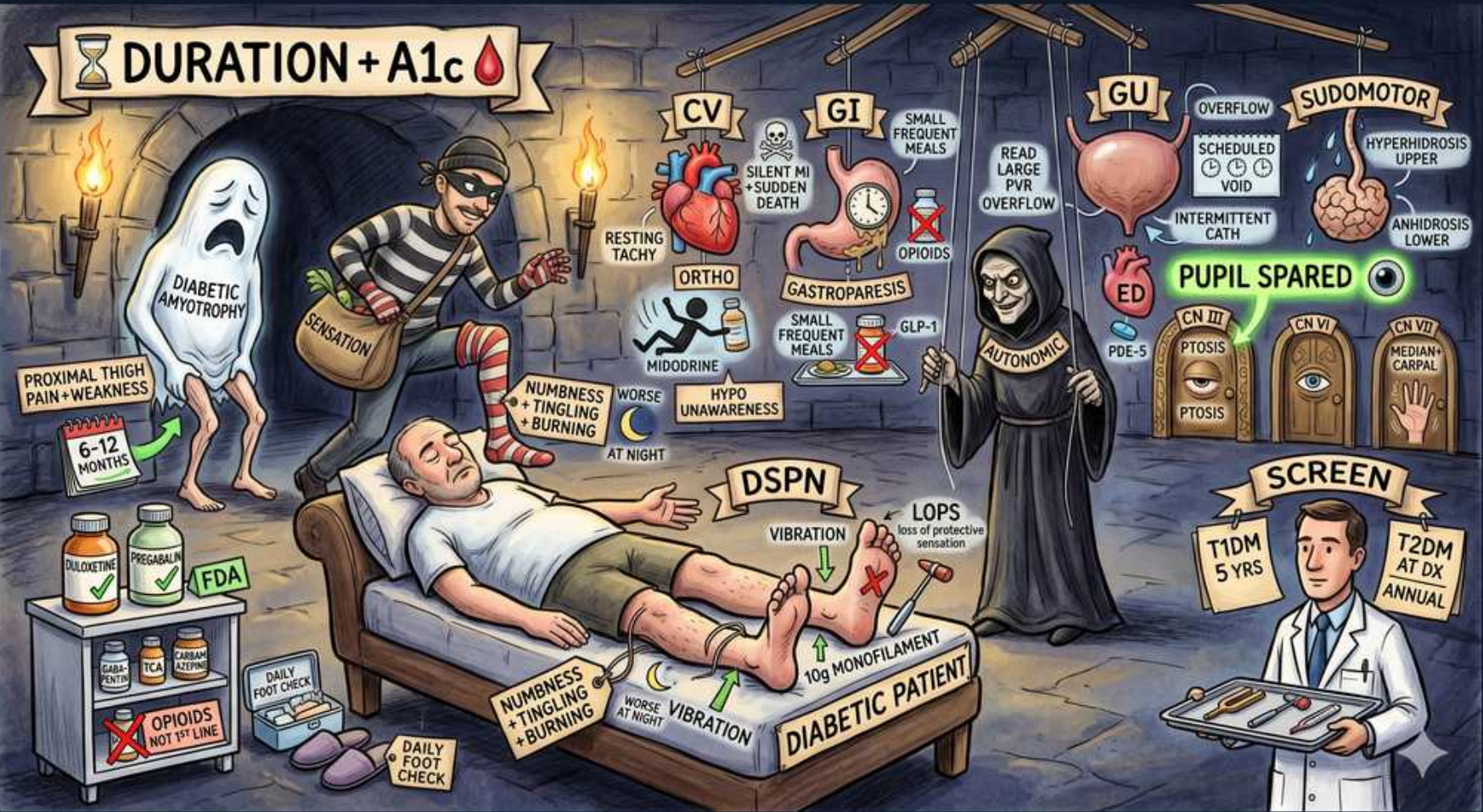


DM — Scene 3 — Diabetic Complications: The Nerve Cellar



1. Duration + A1c banner

WHAT YOU SEE

Banner DURATION + A1c with hourglass

WHAT IT ENCODES

Like all microvascular complications, neuropathy risk climbs with diabetes DURATION and chronic hyperglycemia (A1c). Tight glycemic control clearly prevents/slows neuropathy in T1DM (DCCT); the effect is more modest in T2DM. Neuropathy is the most common chronic complication of diabetes and underlies most diabetic foot ulcers and non-traumatic amputations.

BOARD TRAP

In T2DM, intensive glucose control has only a MODEST effect on preventing neuropathy compared to T1DM — the trap is assuming aggressive A1c lowering will reverse established neuropathy; once present, treatment is symptomatic, and over-aggressive control risks hypoglycemia.

2. Diabetic amyotrophy ghost

WHAT YOU SEE

Ghost labeled DIABETIC AMYOTROPHY, PROXIMAL THIGH PAIN + WEAKNESS, 6–12 months

WHAT IT ENCODES

Diabetic amyotrophy (lumbosacral radiculoplexus neuropathy / Bruns-Garland syndrome) presents as ACUTE, often asymmetric PROXIMAL thigh/hip pain followed by quadriceps WEAKNESS and wasting, frequently with weight loss. It is immune/microvasculitic, usually MONOPHASIC and self-limited over 6-12 months with supportive care and PT.

BOARD TRAP

Amyotrophy is PROXIMAL and predominantly MOTOR — the trap is confusing it with the distal symmetric SENSORY polyneuropathy (DSPN). Proximal asymmetric weakness with wasting is amyotrophy, not stocking-glove DSPN, and is not a reason to escalate to insulin urgently.

3. Sensation thief / stocking

WHAT YOU SEE

Masked thief stealing SENSATION from the legs

WHAT IT ENCODES

The core of distal symmetric polyneuropathy (DSPN) is length-dependent loss of sensation in a STOCKING-GLOVE distribution — feet first (longest axons), then hands once leg involvement reaches the knees. Small fibers (pain/temperature) and large fibers (vibration/proprioception) are both affected.

BOARD TRAP

An ASYMMETRIC or focal sensory deficit is NOT typical DSPN — the trap is labeling a unilateral foot drop or single-nerve deficit as diabetic polyneuropathy; that pattern suggests mononeuropathy, radiculopathy, or compression instead.

4. Worse at night / numbness-tingling-burning

WHAT YOU SEE

A crescent moon over the patient's feet with the words NUMBNESS + TINGLING + BURNING, WORSE AT NIGHT — the moon = the classic nighttime flare of painful distal neuropathy.

WHAT IT ENCODES

Painful DSPN: symmetric distal NUMBNESS, TINGLING/paresthesias, and BURNING/lancinating pain, classically WORSE AT NIGHT. Positive (painful) and negative (numb) symptoms coexist. Loss of pain sensation is what makes feet vulnerable to unnoticed ulcers.

BOARD TRAP

Numbness with painlessness is dangerous, not reassuring — the trap is assuming a patient with no foot pain is low-risk; LOSS of protective sensation is precisely what predicts ulceration and amputation.

5. DSPN label on bed

WHAT YOU SEE

Patient in bed labeled DSPN, DIABETIC PATIENT

WHAT IT ENCODES

Distal symmetric polyneuropathy is the MOST COMMON form of diabetic neuropathy (~75% of cases). It is a clinical diagnosis; nerve conduction studies are reserved for atypical features. Always exclude other reversible causes of polyneuropathy (B12 deficiency, hypothyroidism, alcohol, uremia, monoclonal gammopathy).

BOARD TRAP

Don't reflexively attribute every diabetic's neuropathy to diabetes — the trap is missing a coexisting B12 deficiency (especially in metformin users, which lowers B12). Check B12, TSH, and other reversible causes before declaring it purely diabetic.

6. LOPS / 10g monofilament / vibration

WHAT YOU SEE

Foot exam labeled LOPS, 10g MONOFILAMENT, VIBRATION 128Hz

WHAT IT ENCODES

Screen for LOSS OF PROTECTIVE SENSATION (LOPS) with the 10-gram Semmes-Weinstein MONOFILAMENT plus one of: 128 Hz tuning fork (vibration), pinprick, ankle reflexes, or temperature. LOPS predicts ulcer and amputation risk; positive screen → annual comprehensive foot exam and protective footwear.

BOARD TRAP

A single normal test does not exclude LOPS — the trap is relying on the monofilament alone; combine it with vibration/pinprick/ankle reflex. Also, the goal of the foot exam is risk stratification for ULCERS, not diagnosing the neuropathy subtype.

7. CV autonomic (resting tachy / silent MI)

WHAT YOU SEE

Skull labeled CV with RESTING TACHY, SILENT MI / SUDDEN DEATH

WHAT IT ENCODES

Cardiovascular autonomic neuropathy (CAN): RESTING TACHYCARDIA (early vagal loss), loss of heart-rate variability, exercise intolerance, and SILENT (painless) myocardial ischemia — raising risk of unrecognized MI, perioperative instability, and SUDDEN cardiac DEATH.

BOARD TRAP

A diabetic with new dyspnea, nausea, or fatigue but NO chest pain can be having a silent MI — the trap is ruling out ACS because there's no typical anginal pain; autonomic neuropathy blunts the pain signal, so get an ECG/troponin.

8. Orthostatic / midodrine

WHAT YOU SEE

Falling figure labeled ORTHO with MIDODRINE

WHAT IT ENCODES

Autonomic neuropathy causes ORTHOSTATIC HYPOTENSION (drop without compensatory tachycardia). Treat non-pharmacologically first (rise slowly, fluids/salt, compression stockings), then MIDODRINE (alpha-1 agonist) or fludrocortisone; droxidopa is another option.

BOARD TRAP

Watch for SUPINE HYPERTENSION when treating orthostasis — the trap is dosing midodrine/fludrocortisone at bedtime; give midodrine during the day and avoid late doses to prevent dangerous supine BP spikes.

9. GI: gastroparesis

WHAT YOU SEE

Clock + stomach labeled GI, GASTROPARESIS, SMALL FREQUENT MEALS, GLP-1 caution

WHAT IT ENCODES

Gastroparesis = delayed gastric emptying → early satiety, bloating, nausea, vomiting, and erratic glucose (mismatched insulin/food). Diagnose with a 4-hour gastric emptying scintigraphy after excluding obstruction. Manage with small, frequent, low-fat/low-fiber meals and prokinetics (metoclopramide short-term; erythromycin).

BOARD TRAP

GLP-1 receptor agonists SLOW gastric emptying and worsen gastroparesis — the trap is choosing/continuing a GLP-1 RA in a diabetic with gastroparesis; pramlintide (also slows emptying) is similarly contraindicated. Also limit metoclopramide duration (tardive dyskinesia).

10. GU: neurogenic bladder

WHAT YOU SEE

Bladder labeled GU, OVERFLOW, LARGE/POOR VOID, INTERMITTENT CATH

WHAT IT ENCODES

Diabetic autonomic (neurogenic) bladder: impaired sensation of fullness → large bladder capacity, incomplete emptying, high post-void residual, and OVERFLOW incontinence with UTIs. Manage with timed/scheduled voiding, double voiding, and clean intermittent catheterization for high residuals.

BOARD TRAP

Overflow incontinence from a hypotonic neurogenic bladder is NOT treated with anticholinergics (oxybutynin) — the trap is treating it like urge incontinence; anticholinergics worsen retention. Confirm with post-void residual; treat by emptying the bladder, not relaxing it further.

11. ED / PDE-5

WHAT YOU SEE

A small sign reading ED with a PDE-5 pill tag — flags erectile dysfunction (an early autonomic sign) treated first-line with a PDE-5 inhibitor.

WHAT IT ENCODES

Erectile dysfunction is an early, common autonomic/vascular manifestation in diabetic men. First-line therapy is a PDE-5 inhibitor (sildenafil, tadalafil); second-line includes intracavernosal injections or vacuum devices.

BOARD TRAP

PDE-5 inhibitors are absolutely CONTRAINDICATED with nitrates (and caution with alpha-blockers) — the trap is prescribing sildenafil to a diabetic with CAD who is on nitrates, risking life-threatening hypotension.

12. Sudomotor (anhidrosis/hyperhidrosis)

WHAT YOU SEE

Brain labeled SUDOMOTOR, HYPERHIDROSIS UPPER, ANHIDROSIS LOWER

WHAT IT ENCODES

Sudomotor (sweat) dysfunction: distal/LOWER-body ANHIDROSIS (dry, cracked, fissured feet that predispose to ulcers/infection) with compensatory UPPER-body HYPERHIDROSIS. Gustatory sweating (facial sweating with eating) is another autonomic clue.

BOARD TRAP

Dry, cracked feet here are an AUTONOMIC sign, not just poor hygiene — the trap is overlooking anhidrosis as a contributor to skin breakdown and ulcer risk; emollients and foot care matter.

13. Pupil spared (CN III)

WHAT YOU SEE

Eyes labeled PUPIL SPARED, CN III, PTOSIS

WHAT IT ENCODES

Diabetic third-nerve (CN III) palsy is classically PUPIL-SPARING: down-and-out eye, ptosis, and diplopia but a REACTIVE pupil. Reason: ischemic microvascular infarction damages the central somatic motor fibers while sparing the peripheral parasympathetic (pupillomotor) fibers that run on the nerve's surface. Usually resolves over weeks-months.

BOARD TRAP

A pupil-INVOLVING (blown, fixed, dilated) CN III palsy is a compressive lesion — a posterior communicating artery ANEURYSM until proven otherwise — and a neurosurgical emergency needing urgent CTA/MRA. The trap is attributing a dilated, unreactive pupil to diabetes; diabetic CN III palsy spares the pupil.

14. Screen sign

WHAT YOU SEE

Doctor with tray labeled SCREEN, T1DM 5 YRS, T2DM AT DX, ANNUAL

WHAT IT ENCODES

Screen for distal symmetric polyneuropathy starting at 5 years after T1DM diagnosis and AT DIAGNOSIS for T2DM (silent prior years), then ANNUALLY thereafter. Also assess for autonomic neuropathy symptoms. Use history plus 10 g monofilament and one additional modality.

BOARD TRAP

Mirroring the nephropathy/retinopathy rule: T2DM is screened at the time of DIAGNOSIS (not 5 years later) — the trap is applying the T1DM '5-year' delay to a T2DM patient, missing existing neuropathy present from undiagnosed disease.

15. Treatment bottles (no opioids)

WHAT YOU SEE

Pill bottles labeled FDA-approved, OPIOIDS NOT 1ST LINE, DAILY FOOT CHECK

WHAT IT ENCODES

First-line for painful DSPN: pregabalin (75-150 mg BID, FDA-approved), duloxetine (60 mg/day, SNRI, FDA-approved), gabapentin (titrate to 1800-3600 mg/day), and TCAs (amitriptyline — caution: anticholinergic, orthostasis, arrhythmia). Add daily foot self-inspection and well-fitted footwear to prevent ulcers.

BOARD TRAP

OPIOIDS (including tramadol/tapentadol) are NOT first-line for diabetic neuropathic pain — the trap is selecting an opioid for refractory burning feet; they carry dependence/overdose risk with limited long-term benefit. Optimize gabapentinoids/SNRIs/TCAs first.
